

FORMAL MEDICAL BILLING DISPUTE LETTER

CONFIDENTIAL -- Prepared by Medical Bill Error Finder Agent

PARTIES

FROM (Patient / Sender):

John Martinez
[Your Street Address]
[City, State ZIP]
[Your Phone Number]
[Your Email Address]

TO (Billing Department):

Billing Dept - Dallas Regional Medical Center
[Hospital Street Address]
[City, State ZIP]
[Billing Phone]

Date: April 14, 2026

RE: FORMAL BILLING DISPUTE -- Date of Service: April 3, 2026 - April 5, 2026 | Total Billed: \$11,353.00

NOTICE OF DISPUTE

Dear Billing Department,

I am writing to formally dispute billing errors identified on my itemized statement for services rendered on April 3, 2026 - April 5, 2026. A thorough audit of my hospital bill and Explanation of Benefits (EOB) has revealed 8 billing error(s) resulting in an estimated overcharge of \$4,919.00.

Each error is documented below with the specific CMS rule, NCCI edit, or regulation that has been violated. I request a written response and corrected bill within 30 days, as required under the No Surprises Act (Pub. L. 116-260) and applicable state consumer protection regulations.

DISPUTED ITEMS (8 error(s) identified)

Error #1: UNBUNDLING -- Overcharge: \$850.00

Line Item:	Line 001 - CPT 99213, Office/Outpt Visit Low-Mod Complexity E/M, \$850.00 (04/03/2026)
Description:	CPT 99213 and CPT 99215 were both billed on 04/03/2026 by the same provider (Dr. Kevin L. Nguyen, NPI: 2109876543) at the same facility for the same encounter. CMS NCCI rules [EM-001] permit only one E/M code per day per provider per facility. The lower-level code 99213 must be removed entirely; only the highest-level E/M code (99215) is billable. The \$850.00 charge for 99213 represents an improper duplicate E/M submission resulting in an overcharge of \$850.00 billed (or \$620.00 at the allowed amount, with \$124.00 in improper member responsibility).
Est. Overcharge:	\$850.00
Rule Violated:	CMS NCCI Rule [EM-001]; CMS Claims Processing Manual, Chapter 12, Section 30.6.7 - Only the highest-level E/M code may be billed per day per provider per facility.

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Error #2: UPCODING -- Overcharge: \$1,250.00

Line Item:	Line 002 - CPT 99215, Office/Outpt Visit High Complexity E/M, \$1,250.00 (04/03/2026)
Description:	CPT 99215 is defined as an established patient office or outpatient visit requiring high medical decision making (MDM), typically 40-54 minutes. However, this patient was admitted to the Cardiac Care Unit on 04/03/2026 for an acute MI (I21.9) - this is a hospital inpatient admission, not an office or outpatient visit. The correct billing for the initial encounter on the day of hospital admission would be an initial hospital inpatient care code (99221-99223), not an outpatient E/M code. Billing 99215 for an inpatient admission is a misrepresentation of the place of service and nature of the service. Additionally, this code is being billed alongside 99213 (unbundling, see Line 001). If any E/M is appropriate for 04/03/2026 at the hospital level, it should be 99222 or 99223 (initial inpatient care), not 99215.
Est. Overcharge:	\$1,250.00
Rule Violated:	CMS Claims Processing Manual, Chapter 12, Section 30.6.7 (E/M place of service rules); CMS NCCI Rule [EM-001]; AMA CPT guidelines - 99215 is restricted to office/outpatient settings; initial hospital care must be billed under 99221-99223 per CMS Claims Processing Manual Chapter 12, Section 30.6.8.

Error #3: UPCODING -- Overcharge: \$480.00

Line Item:	Line 004 - CPT 71046, described as 'ROUTINE BLOOD PANEL - METABOLIC SCREEN', \$480.00 (04/03/2026)
Description:	CPT 71046 is the official CMS code for 'Radiologic examination, chest; 2 views' (a chest X-ray). However, the description on this line item reads 'ROUTINE BLOOD PANEL - METABOLIC SCREEN,' which is a laboratory service, not a radiology service. There is a clear mismatch between the CPT code and the billed description. Furthermore, \$480.00 is grossly excessive for a 2-view chest X-ray (typical allowed ~\$50-\$100), suggesting the price may correspond to a metabolic panel or other lab service rather than the stated CPT code. Either (a) the wrong CPT code was applied to a lab service that should have been billed under a panel code (possibly 80053 or 80048, both of which are already separately billed on Line 011), or (b) the description is fictitious and the code was intentionally chosen to inflate reimbursement. Both scenarios constitute a billing violation. The EOB processed this as 'Chest X-Ray, 2 Views' at an allowed amount of \$350.00, meaning BCBS paid based on an incorrect code.
Est. Overcharge:	\$480.00
Rule Violated:	CMS NCCI Policy Manual Chapter 6 - Radiology (CPT 71046 is chest X-ray, 2 views only); AMA CPT Code Set - CPT codes must accurately reflect the service rendered; CMS Claims Processing Manual, Chapter 1, Section 30.3 - Services must be billed with correct CPT codes matching the actual service provided; 42

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CFR §1001.701 (False Claims Act implications for intentional miscoding).

Error #4: INCORRECT QUANTITY -- Overcharge: \$950.00

Line Item:	Line 017 - Room & Board CCU Night 1 (\$950.00), Line 018 - Room & Board CCU Night 2 (\$950.00), Line 019 - Room & Board CCU Night 3 (\$950.00)
Description:	The patient was admitted on April 3, 2026 at 11:42 AM and discharged on April 5, 2026 at 2:15 PM - a documented 2-night stay (the nights of April 3 and April 4). Room & Board has been billed for 3 nights at \$950.00 each (\$2,850.00 total), but only 2 nights (\$1,900.00) are clinically and administratively supported by the admission and discharge records. The third night charge (Line 019, \$950.00 on 04/05/2026) is erroneous. The EOB has processed and paid for all 3 nights, meaning BCBS paid \$576.00 for the invalid third night and the patient was assessed \$144.00 in coinsurance for a night they were not inpatient. The overcharge is \$950.00 billed (\$720.00 at allowed rate, with \$144.00 in improper member coinsurance).
Est. Overcharge:	\$950.00
Rule Violated:	CMS Claims Processing Manual, Chapter 3, Section 20.1 - Inpatient room and board charges must correspond to actual inpatient days; CMS Conditions of Participation 42 CFR §482.13(e) - Hospital must maintain accurate records of patient census; OIG Compliance Program Guidance for Hospitals (63 Fed. Reg. 8987) - Billing for services not rendered or not medically necessary is a false claim.

Error #5: BALANCE BILLING -- Overcharge: \$485.00

Line Item:	Patient Balance Billed: \$2,180.00 vs. EOB-Calculated Patient Responsibility: \$1,695.00 - Discrepancy: \$485.00
Description:	Dallas Regional Medical Center is an in-network provider under the patient's BlueCross BlueShield of Texas PPO Platinum Plan. The EOB clearly calculates the member's total responsibility as \$1,695.00 (20% coinsurance of the allowed amount after contractual discounts). However, the provider's bill demands \$2,180.00 from the patient -- \$485.00 more than the EOB-calculated responsibility. Under the provider's participation agreement with BCBS of Texas, balance billing of the patient beyond their contractual coinsurance is expressly prohibited for in-network providers. This \$485.00 excess charge constitutes illegal balance billing under both the federal No Surprises Act and Texas state insurance law. The EOB itself flags this as a 'POTENTIAL BALANCE BILLING VIOLATION.'
Est. Overcharge:	\$485.00
Rule Violated:	No Surprises Act, 42 U.S.C. §300gg-111 (effective January 1, 2022) - Prohibits balance billing by in-network providers beyond cost-sharing amounts; Texas Insurance Code §1271.155 - In-network providers may not bill enrollees

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amounts beyond applicable cost-sharing; BCBS of Texas Provider Participation Agreement - Contractual prohibition on balance billing in-network members beyond EOB-calculated coinsurance.

Error #6: DUPLICATE CHARGE -- Overcharge: \$850.00

Line Item:	Line 001 (99213, \$850.00) AND Line 002 (99215, \$1,250.00) - same provider, same date 04/03/2026
Description:	Two separate E/M visit charges (99213 and 99215) were billed by the same provider (Dr. Kevin L. Nguyen, NPI: 2109876543) on the same date of service (04/03/2026) at the same facility for the same patient encounter. This constitutes a duplicate charge for the same category of service regardless of the different code levels. Only a single E/M code may be billed per provider per day per facility. Both the EOB (Lines 001 and 002) and the bill's own billing notes (ERROR 1) acknowledge this duplication. The lower-level code (99213, \$850.00) must be voided in its entirety.
Est. Overcharge:	\$850.00
Rule Violated:	CMS NCCI Rule [EM-001]; CMS Claims Processing Manual, Chapter 12, Section 30.6.7 - Only one E/M code permissible per day per provider per facility; AMA CPT Guidelines - E/M services for same provider/same patient/same date are not separately reportable.

Error #7: UPCODING -- Overcharge: \$74.00

Line Item:	Line 009 - J1580, Gentamicin Sulfate 80mg injection x2, \$74.00 (04/03/2026)
Description:	HCPCS code J1580 is listed in the bill and EOB as 'Gentamicin Sulfate 80mg injection.' J1580 is the correct HCPCS code for 'Injection, garamycin (gentamicin sulfate), up to 80 mg.' While the code itself is valid, the clinical appropriateness of gentamicin for a patient admitted with acute MI (I21.9) and Type 2 Diabetes (E11.9) with no documented infection, sepsis, or bacterial indication is highly suspect and raises a SERVICE_NOT_RENDERED or medically unnecessary billing concern. Gentamicin is an aminoglycoside antibiotic that is nephrotoxic, particularly in diabetic patients, and would not be standard of care for an acute MI admission absent a specific infectious indication not reflected in the documented diagnoses. This charge should be supported by documented clinical justification tied to an appropriate diagnosis code.
Est. Overcharge:	\$74.00
Rule Violated:	CMS Claims Processing Manual, Chapter 1, Section 30.6.14 - All billed services must be medically necessary and supported by the documented diagnosis; 42 U.S.C. §1395y(a)(1)(A) - Medicare/insurance exclusion of services not 'reasonable and necessary'; OIG Work Plan - Scrutiny of drug charges not consistent with admitting diagnosis.

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Error #8: DUPLICATE CHARGE -- Overcharge: \$480.00

Line Item:	Line 011 - CPT 80053, Comprehensive Metabolic Panel, \$145.00 AND Line 004 - CPT 71046 described as 'Routine Blood Panel - Metabolic Screen', \$480.00 (04/03/2026)
Description:	If Line 004 (billed under CPT 71046 but described as 'Routine Blood Panel - Metabolic Screen') actually represents a metabolic panel or chemistry screen, then it is being billed redundantly alongside Line 011 (CPT 80053, Comprehensive Metabolic Panel) on the same date. If the description on Line 004 is accurate (i.e., it is truly a metabolic screen), it duplicates Line 011 and both cannot be billed together. If Line 004 is truly a chest X-ray (as the CPT code 71046 indicates), then the description is fraudulently wrong (see separate UPCODING error). In either interpretation, a billing error exists on Line 004.
Est. Overcharge:	\$480.00
Rule Violated:	CMS NCCI Rule [LAB-001] - Comprehensive metabolic panel (80053) and component/duplicate tests cannot be billed together; CMS Claims Processing Manual Chapter 12 - Duplicate billing prohibition; AMA CPT Guidelines - Services rendered once may only be billed once per date of service.

TOTAL ESTIMATED OVERCHARGE: \$4,919.00

AUDIT SUMMARY

A comprehensive audit of the Dallas Regional Medical Center bill (Account ACCT-2026-55814) for patient John Martinez (04/03/2026-04/05/2026) identified eight distinct billing errors spanning unbundling, upcoding, duplicate charges, incorrect quantity, and illegal balance billing. The most significant violations include: (1) dual E/M codes 99213 and 99215 billed by the same provider on the same day in violation of CMS NCCI Rule EM-001 (\$850.00 overcharge); (2) both E/M codes billed as outpatient visit codes when the patient was admitted inpatient for acute MI, requiring initial hospital care codes 99221-99223 instead; (3) CPT 71046 (chest X-ray) applied to a line item described as a metabolic blood panel, constituting a clear CPT/description mismatch and potential duplicate of the separately billed 80053; (4) a third night of Room & Board (\$950.00) billed for a documented 2-night stay; and (5) the provider billing the patient \$2,180.00 when the EOB-calculated patient responsibility is \$1,695.00, a \$485.00 balance billing violation prohibited by the No Surprises Act and Texas Insurance Code for this in-network provider. Total estimated overcharges across all identified errors reach approximately \$4,919.00, and the patient is strongly advised to dispute this bill with BCBS of Texas Member Services and request a full itemized review before making any payment.

REQUESTED ACTIONS

1. Immediately suspend any collection activity for disputed amounts.
2. Provide a fully corrected, itemized bill within 30 days.
3. Remove all duplicate, unbundled, and improperly coded charges.

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4. Recalculate patient responsibility using the corrected charges only.
5. If any disputed charge is believed to be valid, provide supporting documentation (medical records, physician order, operative notes).
6. Confirm receipt of this dispute in writing within 10 business days.

LEGAL NOTICE

Please be advised that I am prepared to escalate this dispute to: (a) the Centers for Medicare & Medicaid Services (CMS) via the Medicare Administrative Contractor; (b) my state's Insurance Commissioner; (c) the No Surprises Act independent dispute resolution (IDR) process; and (d) pursue legal remedies under applicable consumer protection statutes if a satisfactory resolution is not provided within 30 days.

Relevant statutes and regulations include:

- * No Surprises Act, Division BB of the Consolidated Appropriations Act, 2021 (Pub. L. 116-260)
- * 45 CFR Part 149 -- Surprise Billing and Transparency Requirements
- * CMS NCCI Policy Manual for Medicare Services (current year)
- * State balance billing prohibition statutes (as applicable)

I expect a written response directed to the address listed above.

Sincerely,

John Martinez

Date: April 14, 2026